SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

CAPOX / XELOX

Gastrointestinal cancers · NCCP 00321 · version 9 · 21-day cycle

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Patient name	DOB	Hospital no.	Date

Treatment context
CAPOX, also called XELOX, combines capecitabine tablets with oxaliplatin infusion. This NCCP protocol covers adjuvant stage III colon cancer, advanced colorectal cancer and adjuvant gastric cancer after D2 surgery. Confirm the treatment aim and duration for your indication.

Schedule
Oxaliplatin on Day 1; capecitabine twice daily on Days 1–14, followed by Days 15–21 without capecitabine. Follow your individual prescription.

1. GENERIC SACT RISKS / DISCUSSION POINTS

- ☐ Purpose, expected benefit and treatment plan explained
- ☐ Treatment may be delayed, modified or stopped because of toxicity
- ☐ Unexpected, rare or serious adverse effects can occur
- ☐ Supportive medicines and urgent contact advice discussed
- ☐ Pregnancy avoidance / contraception / fertility discussed where relevant
- ☐ Bone-marrow suppression / infection risk
- ☐ Anaemia / fatigue and low platelets / bleeding
- ☐ Nausea / vomiting and bowel disturbance
- ☐ Mouth soreness / mucositis and skin changes
- ☐ Allergic / infusion reaction and other organ-specific toxicity

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Capecitabine
ORAL CHEMOTHERAPY

- ☐ Diarrhoea / dehydration
- ☐ Hand-foot syndrome
- ☐ Mouth ulcers / reduced intake
- ☐ Low blood counts and infection
- ☐ Cardiotoxicity including chest pain / ischaemia
- ☐ Severe early toxicity including DPD-related toxicity
- ☐ Renal impairment and medicine interactions

Oxaliplatin
PLATINUM CHEMOTHERAPY

- ☐ Cold-triggered tingling / throat discomfort
- ☐ Persistent sensory neuropathy, sometimes lasting after treatment
- ☐ Allergic / infusion reaction
- ☐ Low blood counts and infection
- ☐ Nausea, diarrhoea and fatigue
- ☐ Rare serious lung or blood complications

Confirm DPD testing, renal review, the exact tablet prescription, missed-dose and interruption instructions, cold precautions and anti-sickness plan.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00321 v9.

SACTCheck v0.83.0
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